



सत्यमेव जयते

NITI Aayog

DRAFT



SECTOR AT A GLANCE 04

Women Health and Child Health in India

Health Sector in India

DRAFT

Sector at a Glance 04

Women Health and Child Health in India

Health Sector in India

The '**Sector-at-a-Glance**' provides an easy-to-read overview and basic understanding of a sector, sub-sector, or thematic area. The facts and insights on the sector are supported by relevant data and best practices from credible government and multilateral sources. The document covers the structure, key trends, opportunities and challenges, government initiatives, and the way forward for the sector.

Suggested Citation:

NITI Aayog (2023). Women and Child Health in India. NITI Aayog Sector-at-a-Glance Series – Health in India, 4, <link>

Copyright © 2023, NITI Aayog, Government of India

All rights reserved. This draft document and its contents are internal to NITI Aayog. They cannot be reproduced in any manner either in full or parts, thereof. The draft document is subject to further refinements.

All information, ideas, views, opinions, estimates, suggestions, and recommendations (hereinafter 'content') in this draft "portal resource" should neither be understood as professional advice in any manner nor interpreted as policies, objectives, opinions, or suggestions of NITI Aayog. Readers are advised to use their discretion and seek professional advice before taking any action or decision based on these contents. The contents in this portal have been obtained or derived from reliable sources as far as possible. But, NITI Aayog neither represents this information to be accurate nor assumes any liability for any loss, or damages, caused due to any reason whatsoever, towards its usage by a third party. Readers of this document are made aware that the document may be subject to revisions. Any suggestions/input may please be sent to nitiforstates@gov.in.

www.niti.gov.in

Table of Contents

Sr. No	Content	Page No
1	Overview	7-12
2	Key Trends in Women and Child Development Sector	13-19
	Reproductive Health	13-14
	Maternal Health	15
	Newborn Health	16
	Child Health	17
	Adolescent Health	18
	Social Determinants	19
3	Opportunities and Challenges	20-22
4	Cross-Cutting Themes	23
5	Ministries and Departments	24
6	Government Interventions	25-30
7	Way Forward	31
8	Best Practices	32-35
9	Recommended Readings	36

List of Figures

Figure	Page No
Fig 1a - Percentage of currently married women aged 15-49 years who use any modern method of family planning	13
Fig 1b - Unmet Need for Family Planning (currently married women age 15-49 years)	13
Fig 1c - Percentage of women using hygienic menstrual methods	14
Fig 1d - Percentage of women using hygienic menstrual methods by number of years of schooling	14
Fig 2a - Maternal Mortality Rate (MMR) - Deaths Per 100,000 Live births	15
Fig 2c - Maternity Care Indicators	15
Fig 3a - NNMR, IMR and U5MR (Per 1,000 Live Births)	16
Fig 3b - Sample Registration System Reports (2010-13)	16
Fig 4a - Immunisation coverage is highest for BCG vaccine and lowest for third dose of polio among children aged 12-23 months	17
Fig 4b - Nutritional Status of Children	17
Fig 5 - Education and Internet Usage Among Men and Women as per NFHS-5	18
Fig 6 – Primary Health Centres	32
Fig 7 – Beneficiaries	32
Fig 8 – RBSK nurses at community centres	33
Fig 9 – Training under MANSI Initiative	34
Fig 10 - Maternity care at ICU	35

List of Table

Table	Page No
Table 1 - Child Health Goals under NHP 2017 and SDG (2016 – 2030)	16

Overview

Women and children account for about 67.7% of the country's population. Ensuring the empowerment and well-being of these groups is crucial in promoting sustainable and equitable development within the nation. Ministry of Women and Child Development is the major governing body in the Indian government for creating and implementing policies that promote the advancement of women and children. It was established as an independent Ministry on 30th January 2006, having previously operated as the Department of Women and Child Development, which was established in 1985 under the Ministry of Human Resource Development. Sustainable Development Goals (SDGs) have assumed a central role in establishing developmental priorities, with SDG 5 specifically dedicated to the pursuit of gender equality.¹

Ministry of Women and Child Development works closely with the Ministry of Health and Family Welfare on issues related to reproductive, maternal and child health, nutrition, and adolescent health.¹

Background of Ministry of Women and Child Development



Vision

Promoting social and economic empowerment of women and ensuring the development, care, and protection of children through cross-cutting policies and programmes, creating awareness about their rights, and facilitating institutional and legislative support for enabling them to realize their human rights and develop to their full potential.¹



Mission

Empowered women living with dignity and contributing as equal partners in development in an environment free from violence and discrimination. And, well-nurtured children with full opportunities for growth and development in a safe and protective environment.¹



In the last two decades, India has achieved significant advancements in enhancing maternal and newborn health results. Maternal Mortality decreased from 327 to 103 per 1,00,000 live births between the years 2000 and 2018, while neonatal mortality decreased from 44 to 23 per 1,000 live births during the same period. India's reduction in mortality rates exceeded the global and regional declines, irrespective of whether we account for economic growth.²

Gender exerts a significant influence on health, stemming from both biological distinctions and gender-related disparities. The well-being of women and girls is of particular concern, as many societies subject them to discrimination rooted in sociocultural factors. This discrimination manifests in various ways, such as increased vulnerability to HIV/AIDS. Several sociocultural factors hinder women and girls from accessing quality healthcare and achieving optimal health, including:

- Imbalanced power dynamics between men and women.
- Social norms that limit educational and employment opportunities.
- An exclusive emphasis on women's reproductive roles.
- The potential for or actual experience of physical, sexual, and emotional violence.

While poverty poses a significant obstacle to positive health outcomes for both genders, it disproportionately affects the health of women and girls. Factors like malnutrition due to feeding practices and the use of unsafe cooking fuels contribute to this increased burden on their health.³

¹ Ministry of Women and Child Development. (2023). *Annual Report 2022-23*.

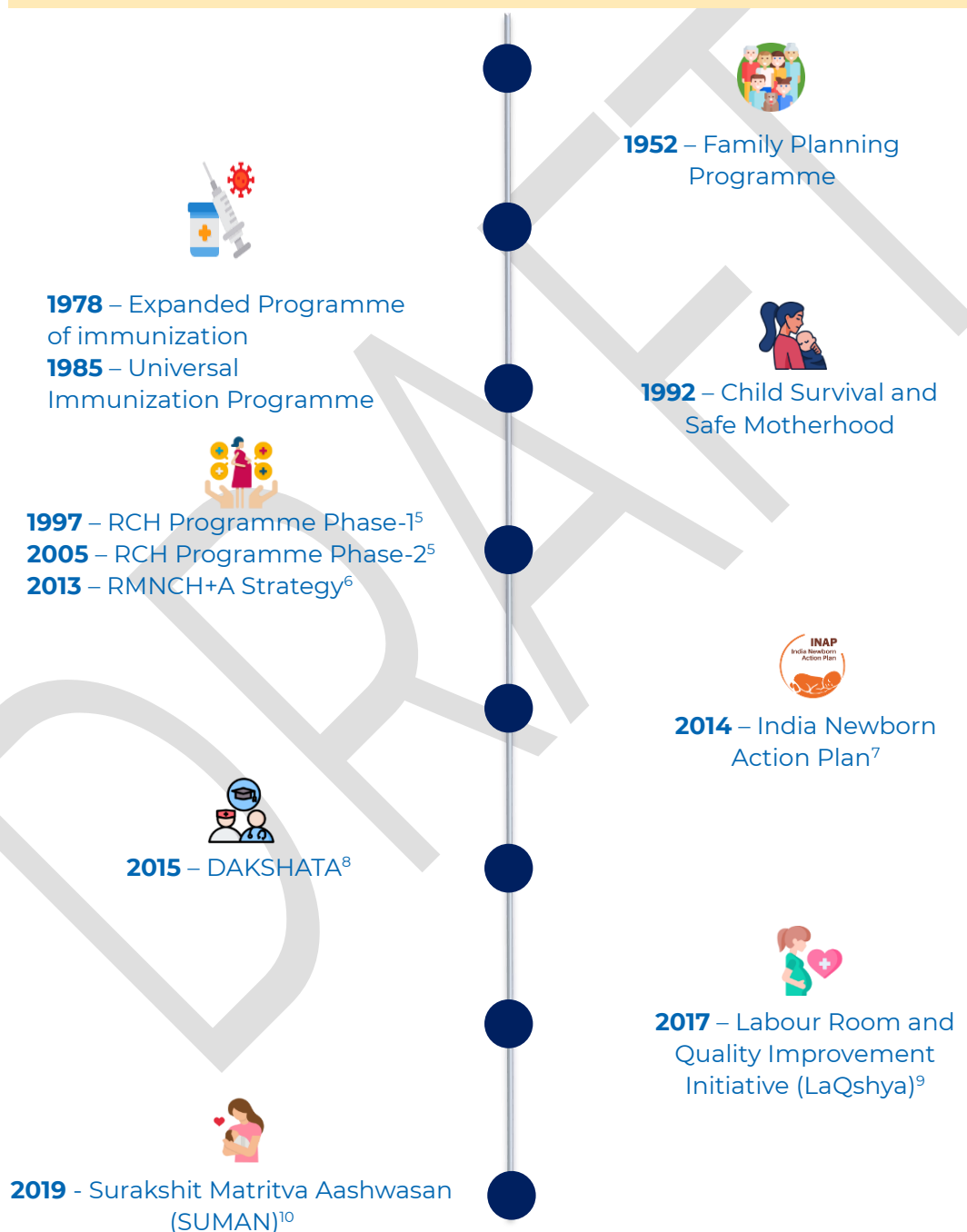
² National Health Systems Resource Centre. (2023). *Exemplars in Maternal and Newborn Health India Study*.

³ World Health Organization. (n.d.). *Women's health*. Retrieved 18 September, 2023.

Evolution of Women and Child Health Programmes

Ministry of Health and Family Welfare and Ministry of Women and Child Development are key stakeholders responsible for driving implementation of programmes and interventions related to women and child health. Maternal and child health (MCH) services were first initiated in the early 1900s in India, when maternity services were improved, and rural midwives and birth attendants received training. In 1968, the Committee on Child Welfare Programmes associated successful family planning (FP) with good MCH services. The five-year plan for 1969-1974 was the first to integrate FP with MCH. India's Integrated Child Development Services (ICDS) programme offers nutrition and health services to pregnant and lactating mothers and young children. The programme has undergone policy reforms that have increased coverage at a national level and reached marginalized groups.⁴

Key programmes under Ministry of Health and Family Welfare (MoHFW)



⁴ Singh S. (1997). Maternal and child health services in India -- past, present and future.

⁵ Priyadarshini. (2019). A Study on Reproductive and Child Programme - Phase I and Phase II.

⁶ Ministry of Health and Family Welfare. (2013). *A strategic approach to Reproductive, Maternal, Newborn, Child Plus Adolescent Health (RMNCH+A) in India*.

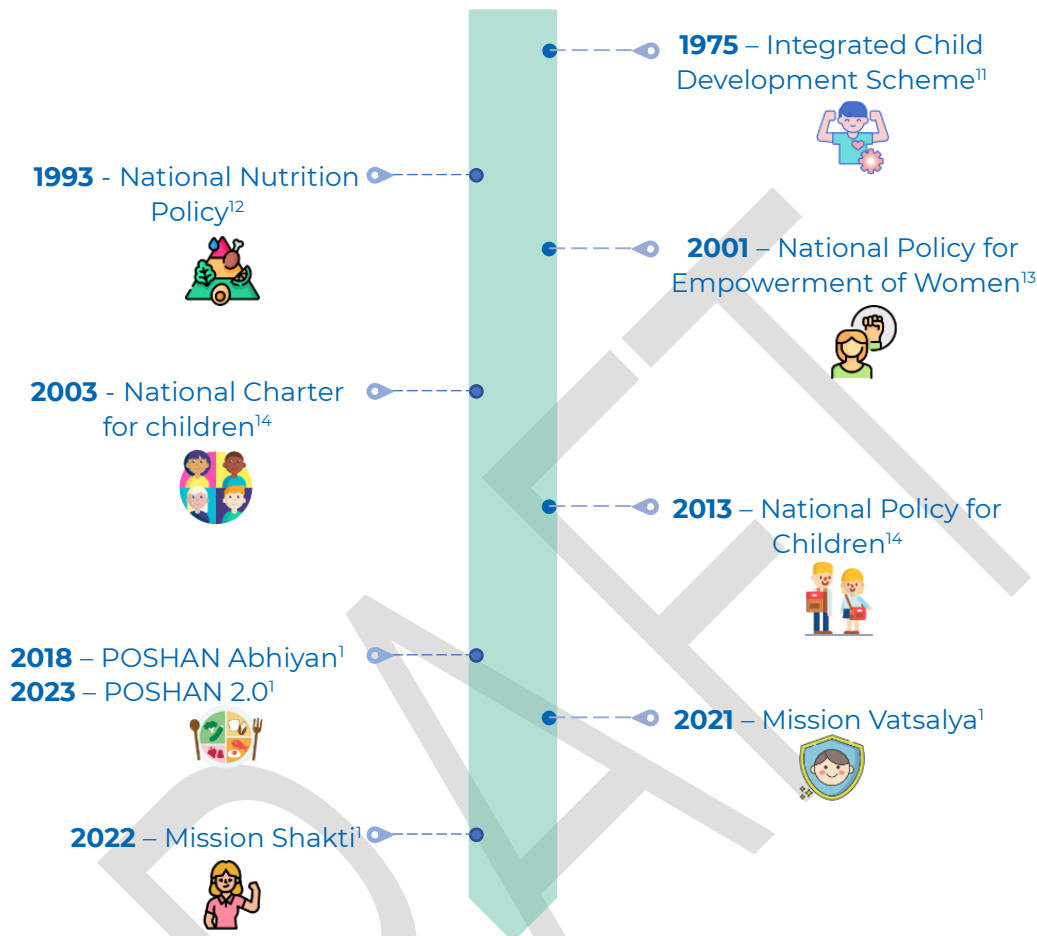
⁷ Ministry of Health and Family Welfare. (2014). *India Newborn Action Plan*.

⁸ Ministry of Health and Family Welfare. (2015). *DAKSHATA*.

⁹ Ministry of Health and Family Welfare. (2017). *LAQSHAYA*.

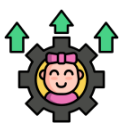
¹⁰ National Health Systems Resource Centre. (2020). *SUMAN Guideline*.

Key programmes under Ministry of Women and Child Development



India has made significant strides in developing government programmes aimed at promoting the well-being and empowerment of women and children. These initiatives have evolved over the years to address various social, economic, and health challenges faced by these vulnerable groups. National Policy for Children, National Nutrition Policy, and the National Charter for Children document provide comprehensive framework for the holistic development and protection of children. Similarly, National Policy for Empowerment of Women, framework for the empowerment of women in India to guide and inspire various sectors and stakeholders. These programmes, among others, demonstrate India's commitment to addressing the unique needs and challenges women and children face. Over time, the government has continued to refine and expand these initiatives to create a more inclusive and equitable society, where the rights and well-being of women and children are prioritised. Here is a brief overview of some key historical developments in major government programmes for women and children in India:

Integrated Child Development Scheme (ICDS)



ICDS is one of India's oldest and most extensive programmes for children and women. It was launched in 1975 as a response to the prevailing issues of malnutrition, maternal and child mortality, and poor childcare practices. Integrated Child Development Services aims to provide a package of services that includes supplementary nutrition, immunisation, healthcare, and preschool education to children below the age of six and pregnant and lactating mothers.¹⁵

¹¹ Ministry of Women and Child Development. (n.d.). *ICDS Mission*.

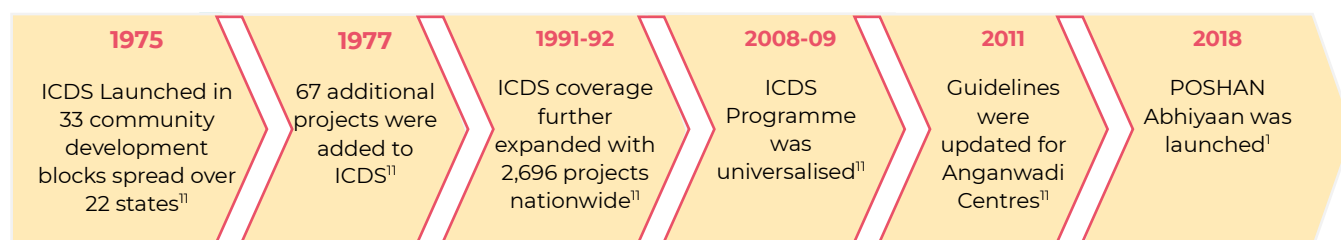
¹² Ministry of Women and Child Development. (1993). *National Nutritional Policy*.

¹³ Ministry of Women and Child Development. (2001). *National Policy for Empowerment of Women*.

¹⁴ Ministry of Women and Child Development. (2013). *Extract from the gazette of India, Part I—Sec. 1, dated 11th May 2013*.

¹⁵ Lal S. (1993). *Past, present and future of Integrated Child Development Services (I.C.D.S.)*.

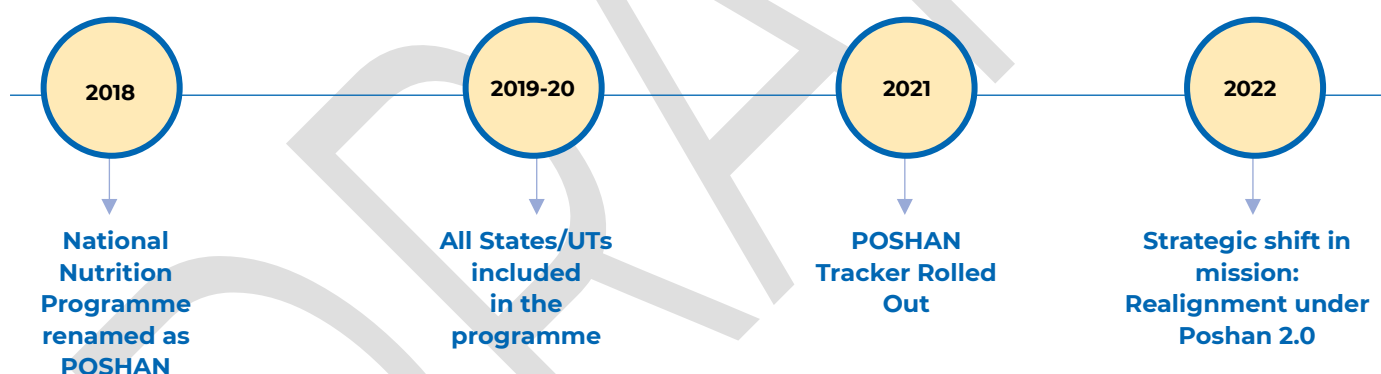
ICDS Timeline



POSHAN Abhiyaan¹⁶



Poshan Abhiyaan is a scheme launched by the Government of India in March 2018 to improve the nutritional status of children (0-6 years), adolescent girls, pregnant women, and lactating mothers. The scheme aims to reduce stunting, under-nutrition, anaemia, and low birth weight babies. It is a multi-ministerial convergence mission with the vision to ensure attainment of malnutrition - free India by 2022. The scheme has been rolled out in all the 36 States/UTs including the state of Tamil Nadu and Odisha. The key components of the scheme include the use of technology, convergence, and behaviour change communication through Jan Andolan. The scheme also aims to provide a platform for convergence of the four pillars



Reproductive, Maternal, Newborn, Child and Adolescent Health (RMNCHA+)²



The RMNCH+A Strategy is framework based on provision of comprehensive care through the five pillars, or thematic areas, of reproductive, maternal, neonatal, child and adolescent health, and is guided by central tenets of equity, universal care, entitlements, and accountability. The aim is to reach the maximum number of people in the remotest corners of the country through a continuum of services, constant innovation, and routine monitoring of interventions.

Launch Dates of Initiatives



RMCH+A 5 X 5 matrix: Maternal and Child Health Integrated Programme (MCHIP) collaborated with Ministry of Health and Family Welfare and other departments to develop the RMNCH+A 5 X 5 matrix to aid in the implementation of the RMNCH+A strategy across the continuum of care. The matrix directs health officials, partners, and programmers in the selection of¹⁷

- Five high-impact interventions across each of the five thematic areas.
- Five cross-cutting and health systems strengthening interventions.
- The minimum essential commodities across each of the thematic areas

Reproductive Health ¹⁷	Maternal Health ¹⁷	Newborn Health ¹⁷	Child Health ¹⁷	Adolescent Health ¹⁷
Focus on spacing methods, particularly PPIUCD at high case load facilities. Focus on interval IUCD at all facilities including subcentres on fixed days. Home delivery of Contraceptives (HDC) and Ensuring Spacing at Birth (ESB) through ASHAS. Ensuring access to Pregnancy Testing Kits (PTK-"Nischay Kits") and strengthening comprehensive abortion care services. Maintaining quality sterilization services.	Use MCTS to ensure early registration of pregnancy and full ANC. Detect high risk pregnancies and line list including severely anaemic mothers and ensure appropriate management. Equip Delivery points with highly trained HR and ensure equitable access to EmOC services through FRUs, Add MCH wings as per need. Review maternal, infant and child deaths for corrective actions Identify villages with high numbers of home deliveries and distribute Misoprostol to selected women in 8th month of pregnancy for consumption during 3rd stage of labour; Incentivize ANMs for	Early initiation and exclusive breastfeeding. Home based newborn care through ASHA. Essential Newborn Care and resuscitation services at all delivery points. Special Newborn Care Units with highly trained human resource and other infra structure Community level use of Gentamycin by ANM	Feeding, IFA supplementation and focus on nutrition. Diarrhoea management at community level using ORS and Zinc Management of pneumonia Full immunization coverage Rashtriya Bal Swasthya Karyakram (RBSK): screening of children for 4Ds' (birth defects, development delays, deficiencies, and disease) and its management	Address teenage pregnancy and increase contraceptive prevalence in adolescents. Introduce Community based services through peer educators. Strengthen ARSH clinics. Roll out National Iron Plus Initiative including weekly IFA supplementation. Promote Menstrual Hygiene

Health System Strengthening¹⁷

- Case load-based deployment of HR at all levels
- Ambulances, drugs, diagnostics, reproductive health commodities
- Health Education, Demand Promotion and Behaviour change communication.
- Supportive supervision and use of data for monitoring and review, including scorecards based on HMIS Newborn Health

Cross Cutting Intervention¹⁷

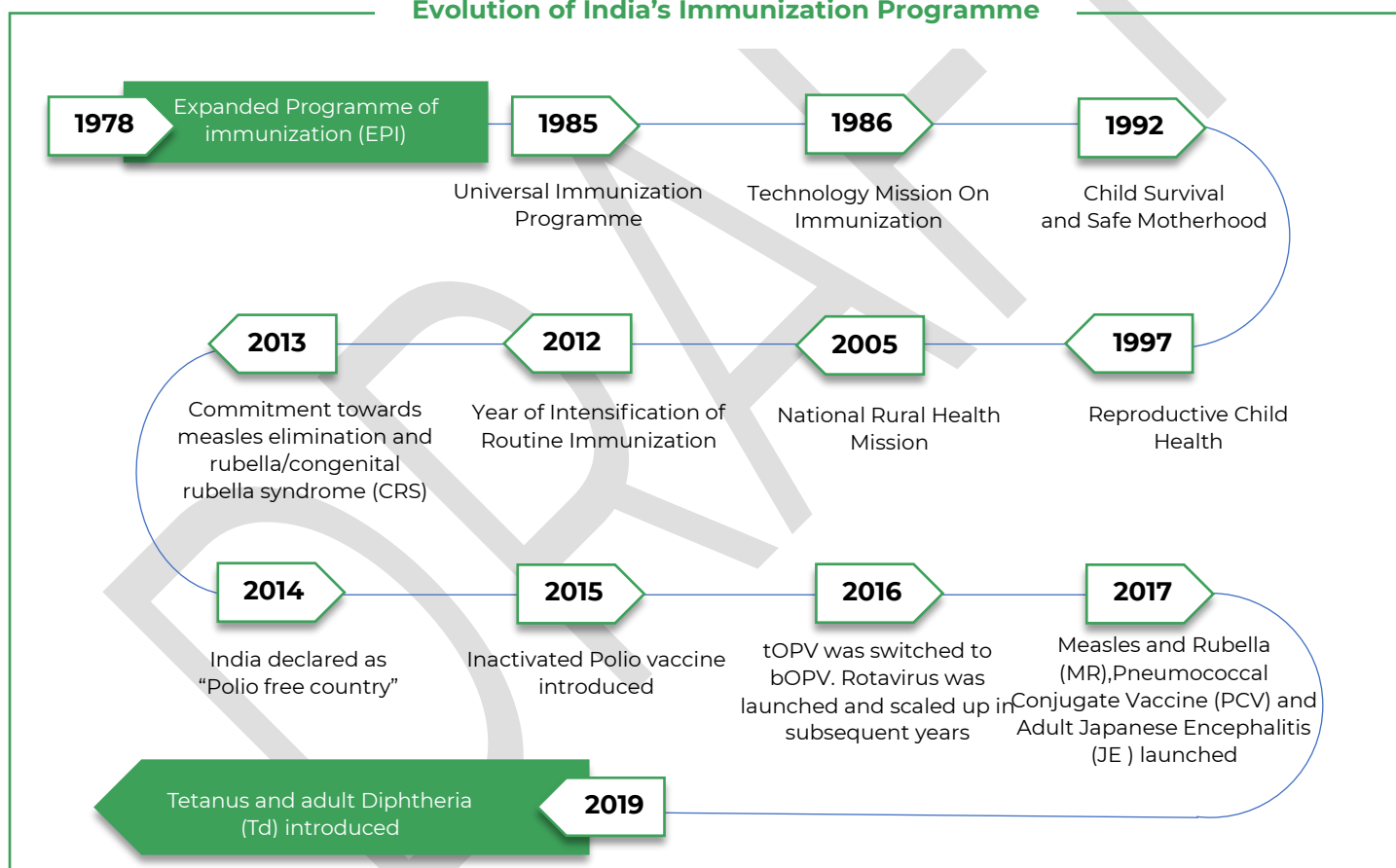
- Bring down out-of-pocket expenses by ensuring JSSK, RBSK and other free entitlements.
- ANMs and nurses to provide specialized and quality care to pregnant women and children.
- Address social determinants of health through convergence.
- Focus on un-served and underserved villages, urban slums and blocks.

Universal Immunisation Programme (UIP)¹⁸



India's immunisation programme is one of the largest public health programmes in the world. Launched as Expanded programme on immunisation in 1978, it was renamed as Universal immunisation programme in 1985 when it was expanded beyond urban areas. Under UIP, immunisation is being provided free of cost against 12 vaccine preventable diseases: Nationally against 11 diseases: Diphtheria, Pertussis, Tetanus, Polio, Measles, Rubella, severe form of Childhood Tuberculosis, Rotavirus diarrhoea, Hepatitis B, Meningitis and Pneumonia caused by Hemophilus Influenza type B and Pneumococcal Pneumonia and sub-nationally against one disease - Japanese Encephalitis.

Evolution of India's Immunization Programme



Introduction of New Vaccines

Rotavirus vaccine (RVV) was introduced in 2016 to reduce mortality and morbidity caused by Rotavirus diarrhoea. In 2019, as per expansion plan, all 25 States/UTs introduced RVV.

Measles Rubella (MR) vaccine was introduced in 2017 through campaign targeting around 41 Crore children in the age group of 9 months to 15 years, followed by 2 doses in routine immunization at 9-12 months and 16-24 months.

Pneumococcal conjugate vaccine (PCV) was launched in a phased manner in UIP in May 2017 and expanded in 2021-22 for reducing infant mortality and morbidity caused by pneumococcal pneumonia.

Tetanus and adult Diphtheria (Td) vaccine was launched. Td vaccine replaced TT vaccine which is given to pregnant woman and children of 10 and 16 years of age from February 2019.

Inactivated Polio Vaccine (IPV) is introduced in UIP as part of Global Polio end-game strategy, to mitigate the risk associated with tOPV to bOPV switch and was expanded across country by 2016.

Trends in Women and Child Development

1. Reproductive Health

Reproductive health is within the framework of the RMNCH+A Strategy of the NHM. It consists of contraceptives, pre- and postnatal care and emergency obstetric and neonatal care, as well as delays in seeking institutional care. Total Fertility Rate (TFR) serves as a crucial measure for assessing the overall reproductive well-being of a community. As the TFR decreases, there is also a reduction in maternal mortality rates. It is associated with both maternal health and the survival of children. TFR is defined as the mean number of children a woman would have throughout her reproductive years.

Fig 1a - Percentage of currently married women aged 15-49 years who use any modern method of family planning¹⁹

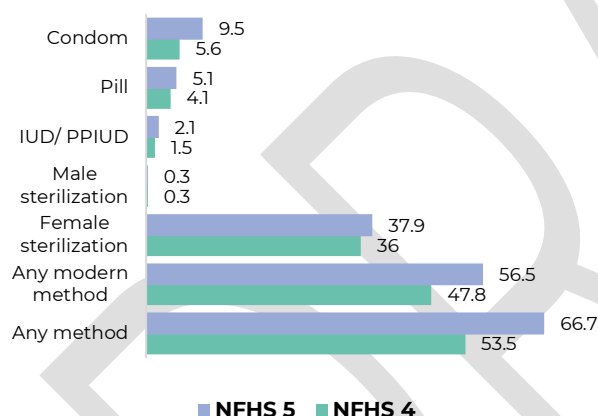
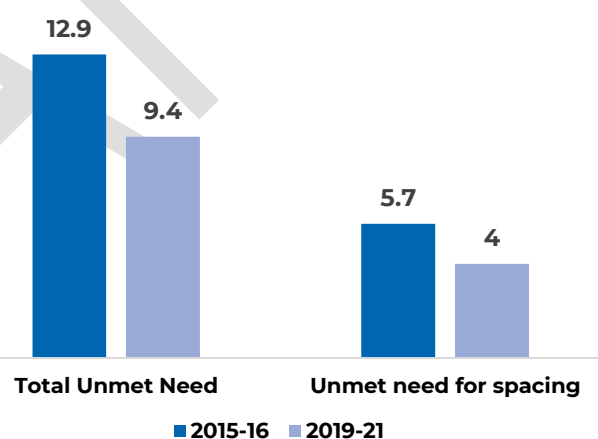


Fig 1b - Unmet Need for Family Planning (currently married women age 15-49 years)²⁰



- ▶ TFR has declined to 2 in 2019-2021 from 2.2 in 2015-2016.²⁰
- ▶ 55.4% of women aged 15 – 49 years are aware about emergency contraceptive method.²¹
- ▶ Modern contraceptive use by currently married women has increased from 48% to 56% between 2015-2016 and 2019-2021.²¹
- ▶ Female sterilization (36.3%) remains notably higher compared to male sterilization (0.2%), despite its greater complexity.²¹
- ▶ Overall contraceptive prevalence rate increased to 67% in 2021 from 54% in 2016. The use of contraceptive in Meghalaya (27%), Mizoram (31%), and Bihar (56%) is low and highest in West Bengal, Odisha, and Himachal Pradesh (74% each) ²¹

¹⁹ Ministry of Statistics and Programme Implementation. (n.d.). *Chapter 2 Health Statistics*.

²⁰ Ministry of Health & Family Welfare. (2022). *Family Vision 2030*.

²¹ Ministry of Health and Family Welfare. (2022). *National Family Health Survey (NFHS-5)*.

DRAFT

²² National Health Mission. (n.d.). *Operational Guidelines Promotion of Menstrual Hygiene among Adolescent Girls*.

Menstrual Health

In India, menstruation and its associated customs are shrouded in societal taboos and cultural constraints that affect both women and adolescent girls. The restricted availability of sanitary hygiene products and the absence of secure sanitary facilities may serve as obstacles to enhanced mobility, potentially leading to the adoption of unsanitary methods for menstrual management. Efforts to promote menstrual hygiene education and awareness are underway.²²

Fig 1c - Percentage of women using hygienic menstrual methods²¹

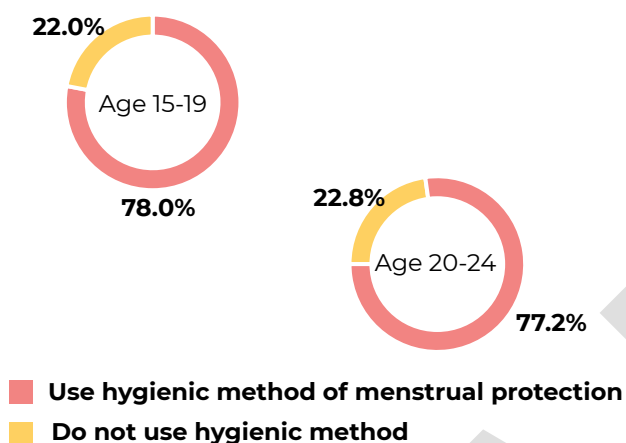
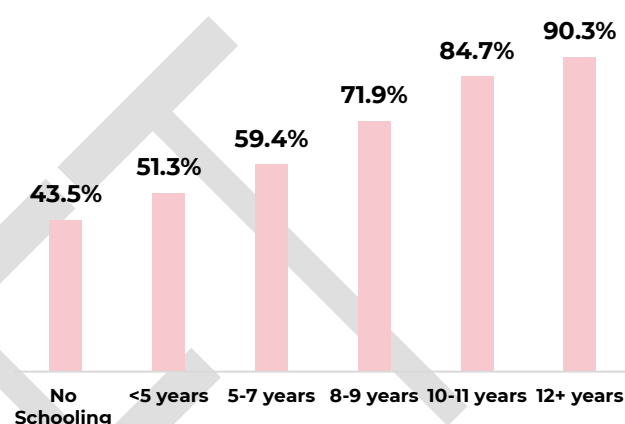


Fig 1d - Percentage of women using hygienic menstrual method by number of years of schooling²¹



- 22% of women aged 15 to 24 years are yet to adopt hygienic menstrual protection methods.²¹
- There is a low usage of sanitary napkins in rural (58.9%) in comparison to urban (77.5%)²¹
- 27% of young rural women still have limited access to menstrual hygiene products.²¹
- Current TFR stands at 2.0 children per woman, marking a decline from 2.2 children in 2015-16.²¹
- Lowest % of women use a hygienic method of menstrual protection in Bihar (59%), Madhya Pradesh (61%), and Meghalaya (65%). In comparison, it is very high in Delhi, Goa, Chandigarh, etc (>90%).²¹

DRAFT

2. Maternal Health

Various efforts have been made by the Government of India, to improve access to antenatal care in pregnancy, skilled care during childbirth, and support in the weeks after childbirth, which has resulted in positive outcome. The number of maternal deaths decreased from an estimated 85,000 to 27,000 during 2000-2018.²

Fig 2a - Maternal Mortality Ratio (MMR) - Deaths Per 100,000 Live births¹⁹

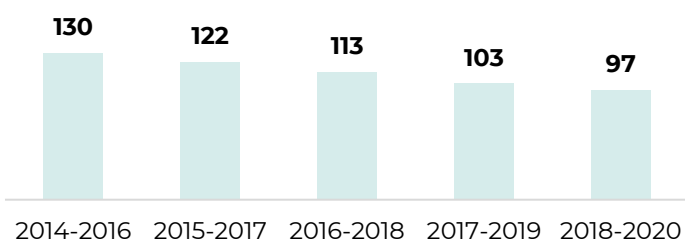


Fig 2b - Trends in Institutional deliveries²¹

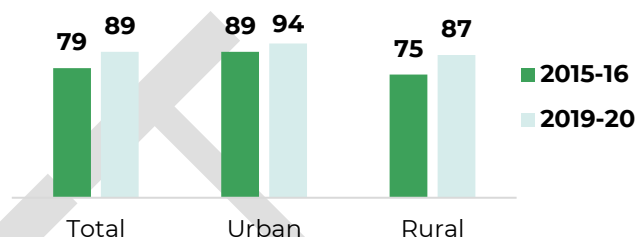
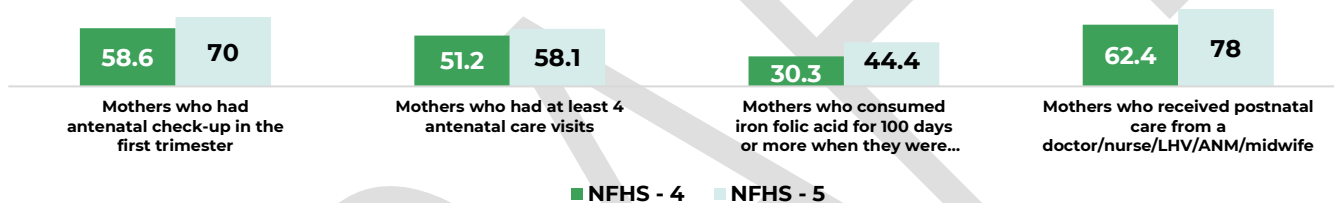


Fig 2c - Maternity Care Indicators in Percentage²¹



- Percentage of institutional deliveries has shown significant growth, rising from 39% in 2005-2006 to 89% in 2019-21.²¹
- 59% adhered to WHO recommendation of having at least four antenatal care visits during their last pregnancy. Conversely, only 6% of women did not have any ANC visits.²¹
- MMR experienced a significant drop, decreasing from 327 maternal deaths per 1,00,000 live births in the period between 1999-2001 to 97 in 2018-2020.¹⁹

3. Newborn Health

In 2013, Newborn and child health became the two key pillars of the Reproductive, maternal, newborn, child and adolescent health (RMNCH+A) strategic approach. Maternal Health Division strives to provide better quality of health services to pregnant women and their newborns through various interventions and programmes.

**Fig 3a - NNMR, IMR and U5MR
(Per 1,000 Live Births)¹⁹**

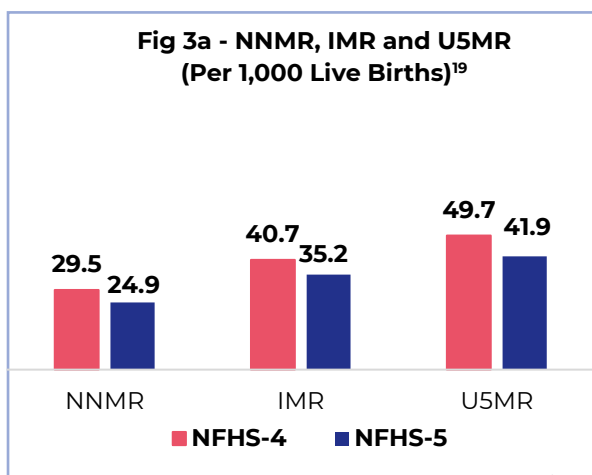


Table 1 - Child Health Goals under NHP 2017 and SDG (2016 – 2030)^{19,23}

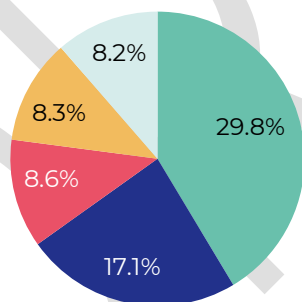
Child Health ²³	Status (NFHS-5) ¹⁹	NHP 2017 ²³	SDG 2030 ²³
Neonatal Mortality Rate	24.9	16 by 2025	<12
Infant Mortality Rate	35.2	28 by 2019	-
Under 5 Mortality Rate	41.9	23 by 2025	=25

NNMR – Neonatal Mortality Rate, IMR – Infant Mortality Rate, U5MR – Under 5 mortality Rate

Leading Causes of Child Mortality

Fig 3b. Sample Registration System Reports (2010-13)²³

- Prematurity and Low Birth Weight
- Pneumonia
- Diarrheal Diseases
- Other NCDs
- Birth Asphyxia and Birth Trauma



Pneumonia and diarrheal diseases are among the leading causes of child death in India.²³

Prevalence of diarrhoea remains more than 4% among children aged less than 60 months and 15% of children do not receive any treatment.²³

DRAFT

²³ National Health Mission. (n.d.). *Child Health*. Retrieved on 16th September 2023.

4. Child Health

The National Health Mission's (NHM) child health programme integrates interventions that promote child survival and address factors that contribute to infant and under-five mortality. Provision of universal immunization of children against vaccine preventable diseases is one of the major goals under National Health Policy.

Fig 4a - Immunisation Coverage is highest for BCG vaccine and lowest for third dose of polio among children aged 12-23 months²¹

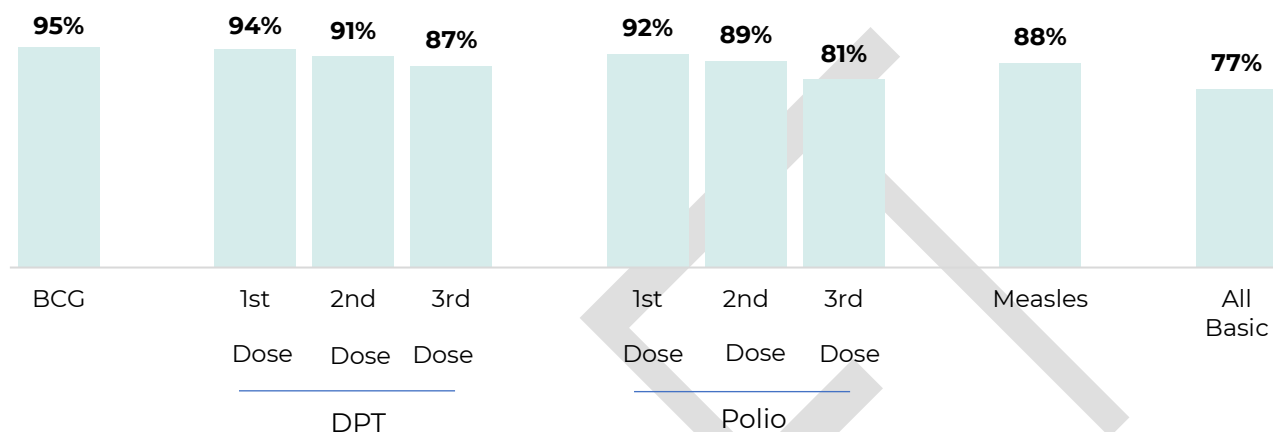
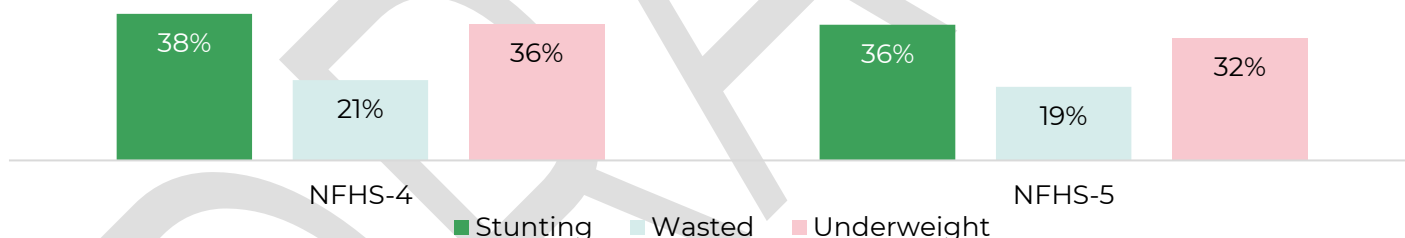


Fig 4b - Nutritional Status of Children²¹



- From 2015-2016 to 2019-2021, there was an increase in anemia prevalence among children aged 6-59 months, rising from 59% to 67%. The issue remained more pronounced in rural children.²¹
- Anaemia caused due to Iron Deficiency is a significant issue that Indian women face with regard to nutrition and prevalence (15-49 years) has increased between NFHS-4 and NFHS-5.²¹
- More stunted children in rural areas (37.3 %) as compared to urban areas (30.1 %).²¹
- The proportion of thin women aged 15-49 decreased from 23% in 2015-2016 to 19% in 2019-2021, while the proportion of overweight or obese women reported increased.²¹

5. Adolescent Health

Adolescent health and nutrition status have an inter-generational effect. Therefore, adolescence is one of the important stages of the life cycle in terms of health interventions. Although adolescence is considered to be a healthy phase, more than 33% of the disease burden and almost 60% of premature deaths among adults can be associated with behaviours or conditions that begin or occur during adolescence – for example, tobacco and alcohol use, poor eating habits, sexual abuse and risky sex (WHO 2002).



7,721 Adult Friendly Health clinics have been setup as of 2023²⁴



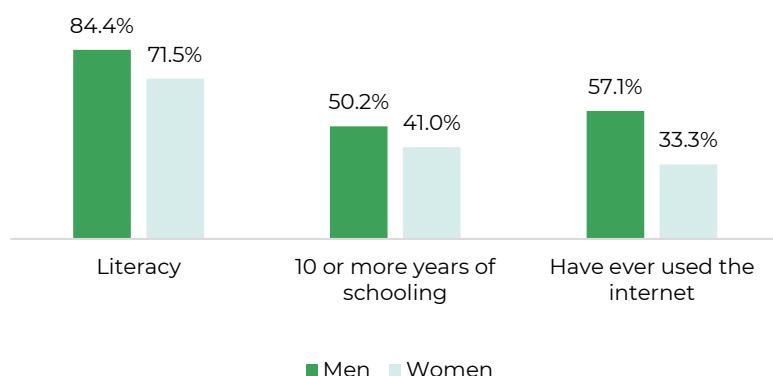
78.23 Lakh adolescents availed services from these AFHCs during 2022-23²⁴

6. Social Determinants

Gender

As per NFHS-5 survey, sex ratio of the total population reached 1020 (per 1000 male population) improving from 991 (per 1000 male population) during 2015-16 survey.²¹

Fig 5 - Education and Internet Usage Among Men and Women as per NFHS-5²¹



78.6%

Women Have a bank or savings account²¹



88.7%

Married women who participate in three household decisions²¹



44.3%

Women own a house and/or land²¹

- Women significantly lag behind men in aspects related to literacy, years of schooling and access to internet.
- Women having a bank or savings account that they themselves use increased from 53% in 2015-16 to 78.6% in 2019-21.²¹
- Percentage of women working (women who worked and got paid in cash) increased slightly from 24.6% in 2015-16 to 25.4% in 2019-21.²¹
- Women land owners also increased from 38.4% in 2015-16 to 44.3% in 2019-21.²¹

²⁴ Ministry of Health and Family Welfare. (2023). *Annual Report 2022-23*.

Domestic Violence

As Per NFHS-5 Report²¹



29.3%

**Ever-married women
age 18-49 years'
experience spousal
violence**



3.1%

**Ever-married women
age 18-49 years'
experience physical
violence during any
pregnancy (%)**



1.5%

**Young women age
18-29 years'
experience sexual
violence by age 18
(%)**

A mere 14% of women who have endured physical or sexual violence from any source have taken steps to seek assistance in putting an end to the violence.¹⁰

Opportunities and Challenges

Opportunities



School Health Programme

- Mid-day meal programme is expanded to include Preparatory Classes in primary school.²⁵
- Health check-ups and growth monitoring from Anganwadi is accessible to both Anganwadi and primary school Preparatory Class students.²⁵



Digital Health

- India has created database of ~24 crore pregnant women and ~20 crore children for monitoring post-natal and immunisation services (data as of 22nd September, 2023)²⁶
- Databases monitoring other health determinants (cancer, malnutrition) could be integrated



Planning

- Between 2016 and 2022, the allotted budget stood underutilized by Ministry of Women and Child Development which indicated opportunity for better planning and monitoring²⁷



Start-ups

- Department of Biotechnology is supporting start-ups like NIRAMAI which is using AI to improve women outcomes in cancer.²⁸
- Incentivizing more such startups will improve child and women health issues.



Integration of last mile delivery

- Poshan Tracker and Mother and Child Health Tracking System promotes real time data with analytics which helps to make informed decisions about resource allocation, programme effectiveness, and policy adjustments.²⁹



Creche Scheme

- It supports working mothers by providing affordable childcare options for their children while they are at work. It encourages more women to join or remain in the workforce. It can help rural women engage in income-generating activities, ultimately improving rural livelihoods.³⁰



POCSO and CARA

- POCSO creates an opportunity for victims to access the help they need to cope with trauma and pursue justice.³¹
- CARA contributes to preventing child abandonment and reducing the number of children in institutional care.³²

²⁵ Ministry of Human Resource Development, 2020. *National Education Policy 2020*.

²⁶ Health and Family Welfare Department, (n.d.). *Reproductive and Child Health Dashboard*. Retrieved 15 September 2023.

²⁷ PRS, 2023. *Demand for Grants 2023-24 Analysis: Women and Child Development*. Retrieved 15 September 2023.

²⁸ Invest India, 2020. *Interview with Geetha Manjunath, Founder and CEO of NIRAMAI Health Analytix Private Limited*. Retrieved 15 September 2023.

²⁹ Poshan Tracker, (n.d.). *Dashboard*. Retrieved 16 September 2023.

³⁰ Ministry of Women and Child Health, 2006. *National Creche Scheme for the Children of Working Mothers*.

³¹ Women and Child Development, 2012. *The Protection of Children from Sexual Offences*.

³² Ministry of Women and Child Development, (n.d.). *About CARA*. Retrieved 16 September 2023

DRAFT

Partnership Based Opportunities

Partnership for Maternal, Newborn and Child Health (PMNCH) is an initiative of WHO, ensuring every child and adolescent in the world is able to realize their right to health and well-being. PMNCH ensures a sustained and equitable focus, on maternal, newborn and child health, including the prevention of stillbirths, with a focus on the poorest and most marginalized communities and during Covid, PMNCH formulated 7 calls to action to strengthen commitment for³⁴:

- Sexual, reproductive, maternal, newborn, child and adolescent health (SRMNCAH) services, supplies, and information and demand generation including for contraception, safe abortion, immunisation, safe delivery, stillbirth prevention, and mental health.
- Advancing sexual and reproductive rights and gender equality.
- Quality care, including respectful and dignified care, and effective community engagement and redress mechanisms.
- Recruitment, training, equal and fair pay, and safe working conditions, including protective personal equipment, for frontline health workers, notably midwives and nurses.
- Social protections, including food and nutrition security, for marginalized and vulnerable groups and enhanced data to better understand and address disparities experienced by adolescents, refugees, the internally displaced, migrants, indigenous communities, persons living with disabilities, among others.
- Functional, safe, and clean toilet and hand washing facilities and quality potable drinking water, with a particular focus on healthcare centres, schools, and centres for refugees and internally displaced persons; and
- Prevention of violence against women, children and adolescents through education and protection programmes.

Key Achievement and Progress Under Partnership for Maternal, Newborn and Child Health (PMNCH)³⁵



Knowledge synthesis

- 9 evidence/progress reports, 10 digital audio and visual knowledge translation material were published.
- 12 media products and 12 advocacy briefs were released. More than 30,000 live presentations were conducted.

- Members increased by 39% since beginning of 2021 and currently 1390 members are present.
- Multiple engagement occurred through 23 minister dialogues and <30 parliamentarians.



Partner Engagement



Campaigns and outreach

- 398 media broadcasted with an estimated potential reach of 6 Billion to drive awareness.
- Increase in followers: 115% on LinkedIn, 6% on Twitter, 7.3% on Facebook and 42% on Instagram.

³⁴ World Health Organization. 2020. *PMNCH Call to Action on COVID-19*. Retrieved 17th September 2023.
³⁵ World Health Organization. 2022. *Achievements and highlights 2022*. Retrieved 17th September 2023.

Challenges

Accessibility



- Women living in rural areas often have limited access to healthcare facilities.
- As many as 60% of women in the country face trouble accessing healthcare for themselves.²¹

Awareness



- Exclusively breastfeeding is practiced by just 64% of infants under six months old, and a mere 11% of children aged 6 to 24 months are provided with a minimum acceptable diet.²¹

Affordability



- Rate of C-sections experienced a significant increase, rising from 8% in 2000 to 22% in 2018. This surge was primarily fueled by the private healthcare sector and an upward trend in C-section rates within public tertiary hospitals.²

Gender Bias



- Women face discrimination and bias in healthcare settings, which prevent them from receiving proper care.

Neonatal Care



- Around 0.6 Million newborns died in India from preterm birth, intra-partum related complications/birth asphyxia, neonatal infections, and congenital malformation in 2018 of which 83% of deaths were preventable.
- In 2018, 3.5 Million premature babies are born, of which 1.7 Million babies are born with birth defects, and one million newborns were discharged from Special New-born Care Units (SNCUs). These newborns remain at high risk of death, stunting, and developmental delay.
- Increase of Special New Born Care Units (SNCUs) in India has abated the rate of neonatal mortality however, more efforts are needed to curb neonatal deaths.³⁶

Malnourishment



- Out of the 22 states/UTs, 13 of them reported an increase in child stunting between 2015-16 and 2019-20. States with increased stunting are Meghalaya, Bihar, Uttar Pradesh, Jharkhand and Gujarat.²¹
- One of the objectives of the POSHAN Abhiyan is to reduce child stunting in 0-6 age group from 38% to 25% by 2022. However, 36% of children under age five are currently affected by stunting (NFHS-2, 2019-2021).²¹

³⁶ UNICEF. (n.d.). *Newborn and child health*. Retrieved 16 September 2023.

Cross Cutting Themes



Climate

Significance: Increase in rainfall is also linked with rise in pneumonia cases in children. Climate change also effect issues like food security and nutrition especially in agriculture dependent rural areas.

Current Efforts: Policies and programmes to strengthen health infrastructure for newborn care have reduced child mortality.

Way Forward: Incorporate One health approach to counteract vector borne diseases especially in rural areas.



Gender

Significance: Gender inequality leads to constraints on movement, a lack of decision-making autonomy, and discriminatory attitudes of healthcare providers.

Current Efforts: Laws for reducing gender discrimination while delivering health services to child, adolescent, and women have been incorporated.

Way Forward: Schemes for working women encouraging more participation in economy like Palna Scheme.



Education

Significance: There is an inverse relationship between years of schooling completed, and rate of mortality of women, newborn, and child.

Current Efforts: Integration of health programmes with school and initiatives like Beti Bachao, Beti Padhao enhance literacy and overall women and child development.

Way Forward: Strengthening educational infrastructure, reducing gender disparities, and promoting comprehensive health education to equip women and children with knowledge for better health outcomes.



Health System Strengthening

Significance: System strengthening ensures accessible, affordable, and equitable healthcare, impacting the overall well-being of the population.

Policy: Strengthening primary healthcare and improving health insurance coverage through Ayushman Bharat. Government is also promoting digital health solutions to enhance healthcare delivery and monitoring.

Way Forward: Ensuring health equity through universal healthcare coverage, for effective healthcare delivery and data management to build a resilient and responsive healthcare system benefiting women and children.

Ministries and Departments



Ministries



Department/ Organisation



Function



स्वास्थ्य एवं
परिवार कल्याण मंत्रालय
MINISTRY OF
HEALTH AND
FAMILY WELFARE

**Ministry of Health
and Family Welfare**

**Department of Health and
Family Welfare**

Department is responsible for family welfare issues, including reproductive health, maternal health, paediatrics, information, education, and communications; collaboration with NGOs and international aid organisations; and rural health services.



महिला एवं
बाल विकास मंत्रालय
MINISTRY OF
WOMEN AND
CHILD DEVELOPMENT

**Ministry of Women
and Child
Development**

**National Institute of Public
Cooperation and Child
Development**

Conducts research and evaluation studies, in the field of public cooperation and child development.

**National Commission for
Protection of Child Rights**

Protects child rights and monitors programmes relating to the survival, welfare, and development of children.

**National Commission for
Protection of Child Rights**

National organisation of India with the mandate of protecting and promoting the interests of women.



Ministry of Consumer Affairs
Government of India

**Ministry of Consumer
Affairs and Food
Distribution**

**Department Of Food and
Public Distribution**

Department is responsible for the management of the food economy of the nation. It also oversees and coordinates various women and child related nutrition related schemes and committed to ensuring that all Indians have access to safe, nutritious, and affordable food.



Government of India
Ministry of Social Justice and Empowerment

**Ministry of Social
Justice and
Empowerment**

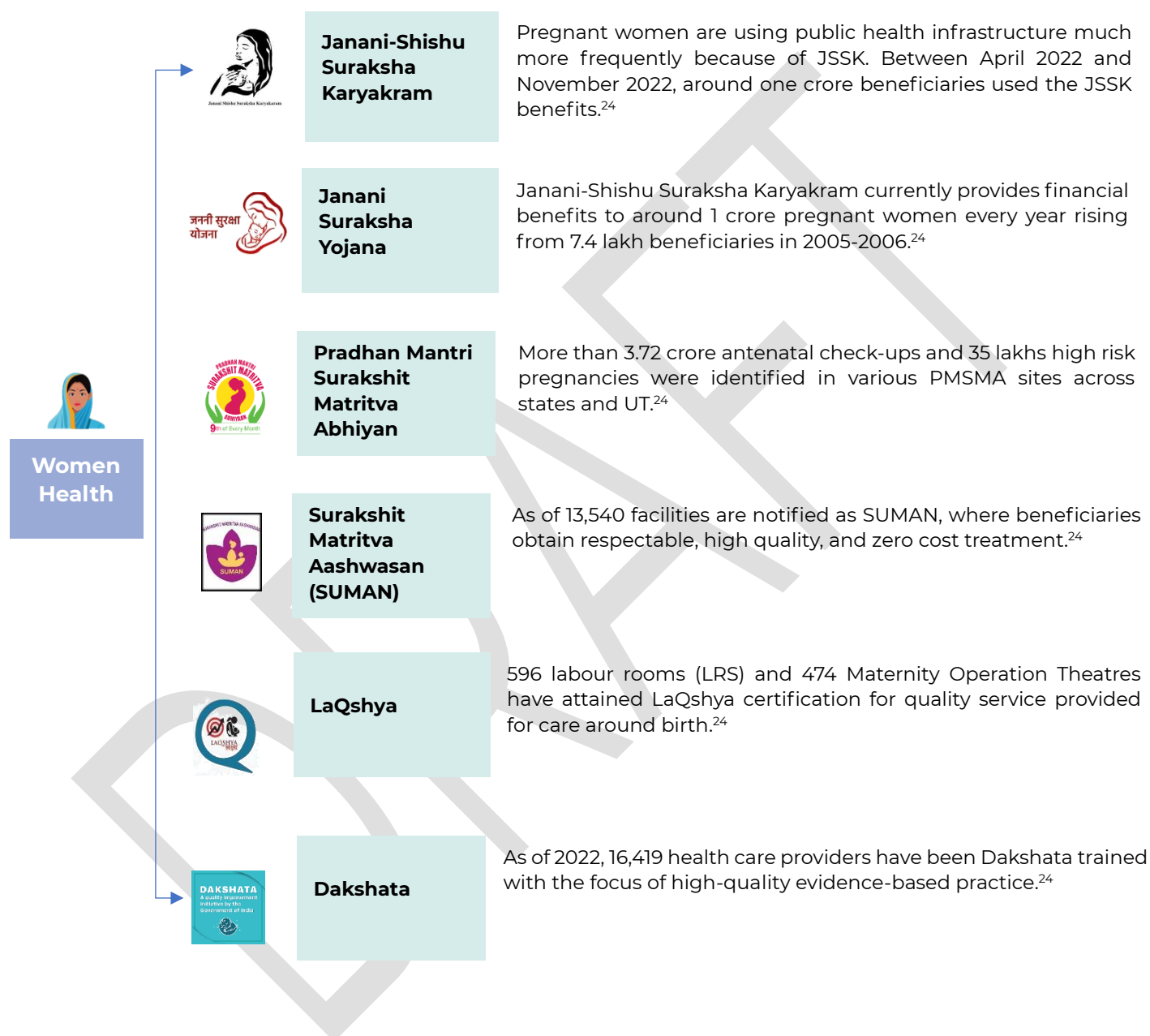
**Department of Social
Justice and Empowerment**

Department is responsible for the welfare, social justice, and empowerment of disadvantaged and marginalized sections of society. The department is committed to working with other stakeholders to ensure that the disadvantaged and marginalized sections of society have equal opportunities and access to resources.

Government Interventions

Prominent national-level programmes are briefly discussed below:

I. Programmes and Interventions under Ministry of Health and Family Welfare





Child Health



Nutrition related interventions



Nutrition Rehabilitation Centres have been established at public health facility level to provide medical and nutritional care to children under five who are severely undernourished (SAM) and have medical issues²⁴

1.02 lakh children were admitted to the NRCs in total in FY 2022-2023²⁴

Lactation Management Centres are being established in public health facilities to ensure availability of safe pasteurised milk²⁴



Bi Annual Vitamin-A supplementation programme, provided 1.3 crore and 1.1 crore children their 1st dose and 9th dose of Vit A supplementation respectively under **Anaemia Mukh Bharat programme**²⁴

Children and adolescents are provided deworming tablet in schools and Anganwadi during **National Deworming Day**²⁴

Infant and Young Child Feeding practices like early initiation of breastfeeding are promoted under **Mother's Absolute Affection programme**²⁴



Pneumonia and diarrhoea



Social Awareness and Actions to Neutralize Pneumonia Successfully (SAANS) seeks to raise awareness against Childhood pneumonia to enhance care seeking behaviours among parents and caregivers.²⁴

Intensified Diarrhoea Control Fortnight (IDCF) Campaign raises awareness about use of ORS and Zinc in diarrhoea. 10.2 crore children up to five years of age were provided with ORS and Zinc in 2022.²⁴



Birth defects, disabilities, delays and deficiencies



Rashtriya Bal Swasthya Karyakram (RBSK) extends the reach of mobile health teams at the Block level to provide child health screening and early intervention services. Initiative aims for early detection of birth defects, diseases, deficiencies, development delays (4 Ds).²⁴

District early intervention centres (DEICs) have been established to provide tertiary level care after screening.²⁴



Maternal and Child Protection Card provides pictorial information related to child development to mothers. MoHFW introduced "Ayushman Bhava" application for wider use of MCP card.²⁴

Paalan 100 (Parenting Advocacy and Learning Advancing Nurturing care in 1000 days) disseminates information about overall development of child and also provides on ground support to care givers.²⁴



immunisation activities



Intensified Mission Indradhanush Drive aims to ensure the consistent delivery of vaccines to target populations.²⁴

Routine immunisation System Strengthening focuses on selected districts and urban areas with low immunisation coverage.²⁴

GoI rolled out an **Electronic Vaccine Intelligence Network (eVIN)** system that digitizes entire vaccine supply chain management. Currently being used in 12 States, the intervention will be scaled to entire country.²⁴

New Vaccines



Inactivated Polio



Rotavirus vaccine



Measles Rubella



Pneumococcal Conjugate



Tetanus and adult diphtheria



Adolescent Health



Rashtriya Kishor Swasthya Karyakram

- 7,721 **adolescent friendly healthy clinics** have been established in which 78.23 lakh adolescents have availed counselling and clinical services during April 2022 to November 2022.²⁴
- 53,686 **adolescent health and wellness days**, were organized with the help of Peer Educators.²⁴
- **Weekly Iron Folic Supplementation programme** is implemented in convergence with Ministry of Education and total of 5.09 crores adolescent were covered during April 2022 to November 2022.²⁴
- **Scheme for promotion of menstrual hygiene** provided 41.21 Lakh Sanitary Napkins during April 2022 to November 2022.²⁴



Ayushman Bharat School Health and Wellness Programme

- **"Health and Wellness Ambassadors"** (HWAS), appointed in schools, receive training to share knowledge on health promotion and illness prevention through engaging activities once a week.²⁴
- In 2022, **27,204 ambassadors were trained**, and 1.29 lakh headmasters were orientated, bringing the total number of HWAs in the nation to 2.42 lakhs.²⁴
- HWAS sessions have gradually started, and in 13 States and UTs, school-age children have now begun participating.²⁴

II. Programmes and Interventions under Ministry of Women and Child Development

Saksham Anganwadi and Poshan 2.0 is an Integrated Nutrition Support Programme. It aims to address the issues of malnutrition in children, adolescent girls, pregnant women and lactating mothers through a strategic shift in nutritional content and delivery and by creation of a convergent ecosystem to develop and promote practices that nurture health, wellness and immunity.¹

Poshan Abhiyan



Community Engagement

- **Jan Andolans** mobilises various communities and sectors to foster a desire to eat nutrient-rich food. Poshan Maah and Poshan Pakhwada, country's two largest nutrition-focused events are held through this initiative.
- **Swastha Balak/Balika Spardha** aims to identify healthy children and raise awareness of the issue and run campaigns to measure young children's growth.
- 4.35 lakh **Poshan Vatikas** have been created at AWCs to promote diet diversification and the use of healthy local produce.¹



Technological Intervention

- **Poshan Tracker** management application offers a 360-degree view of the Anganwadi Centre (AWC) activities, service delivery by Anganwadi Workers (AWWs), and full beneficiary management for expectant mothers, lactating mothers, and children so that results related to women's nutrition can be continuously monitored
- Poshan Tracker has facilitated migration services for expectant and nursing mothers between AWCs inside and outside of a State
- **To ensure last-mile tracking and service delivery**, beneficiaries are being seeded with Aadhar cards. More than 88% of recipients listed on the Poshan Tracker as of December 31st 2022¹



Convergence

- **IFA supplementation, including the supply of IFA tablets, Health check-up** and referral services, and Nutrition and Health Education, are provided by establishing convergence with Ministry of Health and Family Welfare
- immunisation in Palna homes through convergence with Mission Poshan 2.0
- Government intends to construct 50,000 AWCs building at the rate of 10,000 AWCs per year in convergence with MGNREGS under Mission Poshan 2.0
- Increasing awareness of traditional foods for healthy pregnancy and childbirth (especially in tribal areas) in coordination with **the Ministry of Tribal Affairs**¹

Anganwadi Services

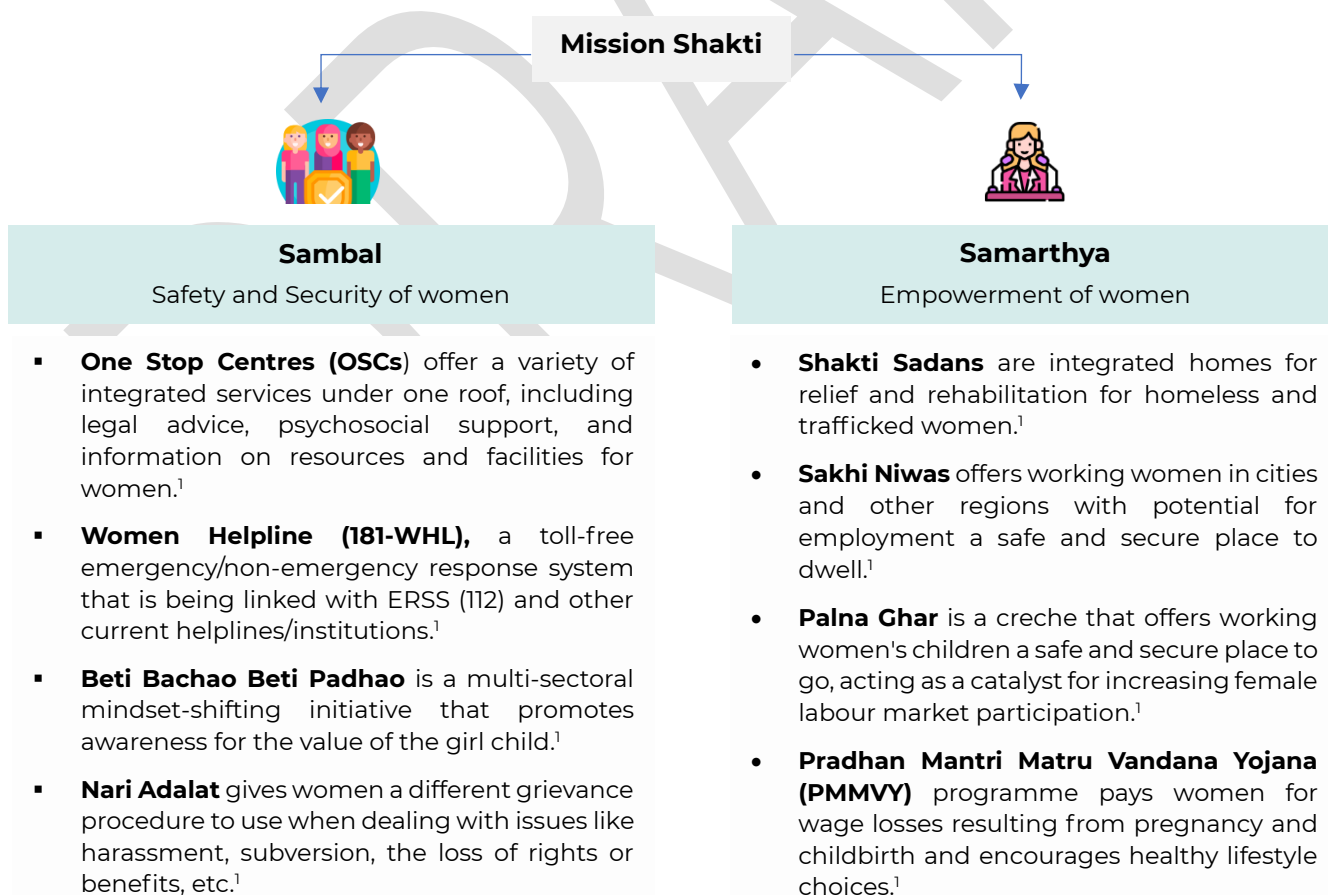
Anganwadi services aim to better the nutritional and physical health of kids between the ages of six months and six years through following services.

Objectives

- To improve the nutritional and health status of children in the age-group of 6 months - 6 years.
- To lay the foundation for proper psychological, physical, and social development of the child.
- To reduce the incidence of mortality, morbidity, malnutrition, and school dropouts.
- To achieve effective co-ordination of policy and implementation amongst various Departments to promote child development
- To enhance the capability of the mother to look after the normal health and nutritional needs of the child through proper nutrition and health education.

As of June 30, 2022, the Anganwadi Services Scheme was running through a network of 7,074 fully functioning Projects and 13.91 lakh operational AWCs. As of June 30, 2022, there were 9.51 crore beneficiaries of the Supplementary Nutrition Programme, including breastfeeding mothers and pregnant women.¹

Mission Shakti: As an umbrella programme for the safety, security, and empowerment of women, the Ministry of Women and Child Development has developed "Mission Shakti," an integrated women's empowerment programme. The components of Mission Shakti have been created in a way that it meets the needs of women throughout their whole life cycle.¹



Mission Vatsalya: offers preventive, statutory care and rehabilitation centers to child in need as per Juvenile Justice (Care and Protection of Children) Act, 2015. It further aims to foster a system of development for children in difficult situation and has multiple components under it.

Components within Mission Vatsalya¹



Statutory Bodies

Child Welfare Committees (CWC), Juvenile Justice Boards (JJB) and Special Juvenile Police Units (SJPU).



Institutional Services

Children Homes, Specialized Adoption Agencies, Open Shelters, Observation Homes, Special Homes, Places of Safety.



Vatsalya Sadan

An Integrated Home Complex of CCIs (Children Home, Observation Home, Special Home, Place of Safety) along with JJB and CWC located within single premises.



Service delivery Structures

State Child Protection Society (SCPS), State Adoption Resource Agency (SARA), District Child Protection Unit (DCPU).



Family based non-institutional care

Sponsorship provided the education, health and nutrition. Foster Care, Adoption services, After Care fall within this component.



Phone services

Emergency outreach phone services for children in crisis/difficult circumstances, through 24 hours toll free number 1098.

Central Adoption Resource Authority (CARA)¹



Objective of CARA is to work for rehabilitation of children through adoption keeping in mind their best interest.

Interventions through CARA

- CARINGS portal was launched in February, 2011 and hosted on the official website of CARA. This web-based monitoring system provides a transparent and child friendly adoption process across the nation.
- CARA has framed the latest Adoption Regulations in 2022 as mandated under section 68 (c) of the Juvenile Justice (Care and Protection of Children) Act, 2015 (Amended in 2021) which has been operational from 23rd September, 2022. The New Adoption Regulations aims to strengthen adoption programme in the country by streamlining the adoption process.

Protection of Children from Sexual Offences (POCSO) Act, 2012



The POCSO Rules were reviewed after the amendments in the POCSO Act, 2012. After careful consideration of all aspects and detailed deliberations, the Protection of Children from Sexual Offences (POCSO) Rules, 2020 were approved and notified, replacing the POCSO Rules, 2012.

Interventions through POCSO

- The Ministry has been coordinating with different Ministries and State Governments/UT Administrations for suitable action, which also aims at greater awareness, speedy justice and better services for child victims under POCSO.
- To facilitate reporting of offences/ complaint, the POCSO e-Box was launched by the Hon'ble Minister for Women and Child Development, in New Delhi, on 26th August, 2016.



Telemedicine For Rural Women Health

- Over 114 million patients were serviced through National Telemedicine Service of India between November 2019 and July 2023.³⁷
- 57% of beneficiaries were women, reflecting the platform's ability to reach the more vulnerable sections of the population.³⁷



Gender Inclusive Policies

- Ayushman Bharat HWCs expanded range of services includes family planning, reproductive health services and Care in pregnancy and child-birth.³⁸
- For working women, initiatives like Palna Ghar have started gaining impetus after slowing down during Covid-19 and 8.68 Crore rupees has been released till date.¹



Dietary Diversity

- About 81% of the Ministry of Women Child Development's budget is allocated to Saksham Anganwadi and POSHAN 2.0 scheme for FY 2023-2024.¹
- Diet diversity and essential micronutrient to address issues of vitamin deficiency will be areas of focus.



Intensification of Immunisation

- Focus can be on increasing immunisation coverage of hard-to-reach areas, unvaccinated and partially vaccinated children and pregnant women.
- 143 districts will see an increase in immunisation efforts as part of a priority to lower the number of zero dose children overall.³⁹



Real Time Tracking of Health Outcome

- POSHAN Tracker enables real-time monitoring and tracking of all AWCs, AWWs and beneficiaries on defined indicators.
- To ensure last-mile tracking and service delivery, beneficiaries are being seeded with Aadhar cards.¹

³⁷ Ministry of Health and Family Welfare. (n.d) eSanjeevani - National Telemedicine Service.

³⁸ Ministry of Health and Family Welfare. (2023). Annual Report 2022-2023.

³⁹ Gavi. (n.d.). Gavi and Government of India establish new partnership to protect millions of children by 2026.

Best Practices

1. Kangaroo Care Project - Uttar Pradesh⁴⁰

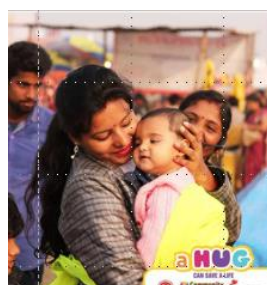
Programme unit to encourage mothers and fathers to maintain skin-to-skin contact with their babies.

1. **Background:** Kangaroo Care is a method of care for newborns, especially low birth-weight babies, that involves skin-to-skin contact between the baby and the mother or father. The Kangaroo Care Project in Uttar Pradesh is a programme unit that encourages mothers and fathers to maintain skin-to-skin contact with their babies to promote bonding and improve health outcomes for the baby.
2. **Steps taken by the Government:** The government of Uttar Pradesh collaborated with an organization in India to support a Kangaroo Mother Care programme to promote the health of newborns. The programme aimed to integrate immediate Kangaroo Mother Care (iKMC) into routine health services in Uttar Pradesh.
 - **Agrimaas in kangaroo care project:** Young female volunteers are taking on important roles, including establishing guidelines for providing respectful care, pinpointing minor yet impactful obstacles to implementation, and driving ongoing enhancements in quality within healthcare facilities. They serve as crucial agents in closing the empathy divide among healthcare staff and fostering better connections between the healthcare system and families.
 - **Empathy:** The decreased morale among health workers and the fostering of feelings of disrespect and dissatisfaction among mothers and families are brought about by lack of empathy between the healthcare system and communities. This gap was addressed and filled to achieve success in healthcare initiatives.
 - **Hug of life campaign** was conducted which a simple and effective method for premature and low birth weight children.
 - **Monitoring and Evaluation:** It typically includes mechanisms for monitoring and evaluating the implementation of kangaroo care. This might involve tracking the number of babies receiving kangaroo care, monitoring health outcomes, and assessing parent satisfaction.
3. **Impact generated (in one year) :**
 - No. of Kangaroo Care facilities: 44
 - No. of Beds: 262
 - No. of Babies initiated on kangaroo care: 1,394
 - Average hours of Kangaroo care per day: 10.4

Fig 6 – Primary Health Care Centre



Fig 7 – Beneficiaries



2. Shalabham Comprehensive Newborn Screening Programme – Kerala⁴¹

A newborn screening programme that includes a comprehensive package of VBD screening, Pulse oximetry, OAE screening and Metabolic screening to detect early birth defects.

1. **Background:** SHALABHAM Comprehensive Newborn Screening Programme is a government initiative in Kerala, India, that aims to screen all newborns born in public health facilities for various birth defects. The program is part of the National Health Mission (NHM) and is in line with the Rashtriya Bal Swasthya Karyakram (RBSK) programme, which aims to provide comprehensive healthcare to children under 18 years of age. Kerala boasts the country's most favourable infant mortality rates, with only 10 deaths per 1,000 live births, among the larger states in India.
2. **Steps taken by the Government:** The SHALABHAM programme has several components, including screening for hearing impairment, congenital heart disease, and metabolic disorders. The screening is done within 48 hours of birth and is free of cost for all newborns born in public health facilities. The programme also provides follow-up care and treatment for infants who are diagnosed with birth defects. It covers all 98 government nodes. (NHM) in Kerala has initiated a comprehensive screening programme for children under the age of eighteen, aligning with the RBSK programme policy. The objective is to detect, refer, and manage the "4 D's" at an early stage. This screening process is conducted at all public sector healthcare facilities across Kerala

The screening process involves multiple stages:

- Screening for Visible Birth Defects (VBD): This assessment is performed within the first 24 hours of a child's life.
- Pulse Oximetry (PO) Screening: Along with a focused physical examination, this screening aims to identify congenital heart diseases (CHD) within 24-48 hours of birth.
- Otoacoustic Emissions (OAE) Screening: This test is conducted after 48 hours but before the child's discharge.

Children identified with birth defects receive treatment from tertiary care hospitals, DEICs (District Early Intervention Centres), specialized centres, etc. Furthermore, they receive follow-up care from RBSK nurses at the community level.

3. Impact generated:

	Month	June	July	August	September	October
Screening Results Reported	VBD Screened	11,147	14,716	13,771	15,054	16,181
	PO screened	3,907	7,890	7,950	8,350	8,107
	OAE Screened	136	2,515	5,979	6,661	3,946
	Metabolic reported	2,253	6,482	7,247	6,727	1,747

Fig 8 – RBSK nurses at community centres



40 National Health Systems Resource Centre
41 National Health Systems Resource Centre

3. Maternal And Newborn Survival Initiative (MANSI) – Jharkhand⁴²

A programme to reduce neonatal and infant mortality by facilitating improved implementation of Home-Based Newborn Care (HBNC) through the existing govt. health machinery at village level (Sahiyas) in a Public private Partnership.

- 1. Background:** High number of deaths of newborn babies within the first month of life occurs during pre-birth, delivery and post-birth stages, most of which are preventable deaths Pneumonia, Asphyxia/prolonged delivery.
- 2. Steps taken by the Government:** The project's primary objective is to decrease neonatal and infant mortality in the Seraikela block of Jharkhand. This will be achieved by enhancing the implementation of Home-Based Newborn Care (HBNC) through the existing government health infrastructure at the village level, with the active involvement of community health workers known as Sahiyas, in a public-private partnership model. The project also aims to strengthen the public health system in this region.

To execute this initiative effectively, the project area has been divided into different zones, with each field coordinator assigned specific responsibilities within their designated area. The strategy includes training Sahiyas, providing Training of Trainers (ToT) sessions at SEARCH, conducting ongoing monitoring and support through a mentorship approach, and ensuring that infrastructure and health resources align with government guidelines, all of which are facilitated by SEARCH.

Furthermore, SEARCH has established a Management Information System (MIS) to closely monitor and guide the coordinators on a daily, weekly, monthly, quarterly, and annual basis to ensure the project's success.

- 3. Impact Generated:** Several case studies overseen by the Sahiyas have been documented, including instances involving infants with low birth weight and hypothermia, as well as cases of pneumonia and sepsis. Through these interventions, there has been a significant reduction in both the infant mortality rate, which has decreased by 46%, and the neonatal mortality rate, which has seen a reduction of 39% from 2011 to 2014.

Despite numerous challenges such as low literacy rates, a shortage of human resources, and limited materials, the Public-Private Partnership (PPP) initiative is planning to expand the programme to districts in Odisha.

Fig 9 – Training under MANSI Initiative



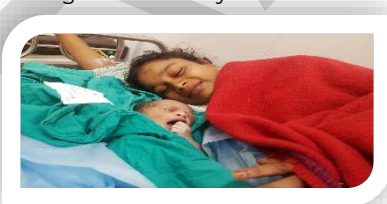
⁴² National Health Systems Resource Centre. 2022. *Best Practices in area of Tribal Health*.

4. Respectful Maternity Care – Maharashtra⁴³

An initiative taken by the government to bring changes in clinical practices at maternity care by providing more care over the child bearing cycle.

- 1. Background:** Respectful Maternity Care (RMC) is an important concept in the field of maternal and reproductive health. It refers to the care and treatment that women receive during pregnancy, childbirth, and the postpartum period that is dignified, culturally sensitive, and respectful of their autonomy and rights. The programme is working on improve quality of care for pregnant women in labour room, maternity Operation Theatre and Obstetrics Intensive Care Units (ICUs) and High Dependency Units (HDUs).
- 2. Steps taken by the Government:**
 - Revamped OPD entrance gate, labour room, ANC clinic
 - Education on wanted or unwanted pregnancy
 - Free voice message services
 - Awareness on intimate partner violence
 - Explained advantages of upright position in first and second stage of labour
 - Position of choice for birthing
- 3. Impact Generated:**
 - By promoting natural birthing positions, the demand for oxytocics and other uterotonics has seen a continuous reduction, dropping from 33% in 2017 to 19% in 2018.
 - Rate of NICU admissions had decreased thanks to the implementation of various practices, including the utilization of a safe childbirth checklist, delayed cord clamping, promoting skin-to-skin contact between the baby and mother, and initiating early breastfeeding within the first half-hour after birth.

Fig 10 – Maternity care at ICU



⁴³ National Health Systems Resource Centre. 2022. *Respectful Maternity Care-Maharashtra*.

Recommended Readings

1. Ministry of Health and Family Welfare. (2022). *National Family Health Survey (NFHS-5)*. http://rchiips.org/nfhs/NFHS-5Reports/NFHS-5_INDIA_REPORT.pdf
2. UNICEF. (n.d.). *Maternal and New Born Health Disparities India*. https://data.unicef.org/wp-content/uploads/cp/maternal-newborn-health/country%20profile_IND.pdf
3. Niti Aayog. (2021). *Poshan Abhiyaan Monitoring, NITI Aayog*. <https://niti.gov.in/sites/default/files/2023-02/Poshan-Abhiyaan-Monitoring.pdf>
4. Ministry of Women and Child Development. (2023). *Annual Report 2022-23*. https://wcd.nic.in/sites/default/files/ar-2-211E_merged.pdf
5. NITI Aayog. (2023). *Best Practices in Social Sector*. <https://niti.gov.in/sites/default/files/2023-05/Best-Practices.pdf>
6. Census India. (2022). *Special bulletin on Maternal mortality in India 2018-20*. https://censusindia.gov.in/nada/index.php/catalog/44379/download/48052/SRS_MR_Bulletin_2018_2020.pdf
7. National Health Mission. (2018). *Operational Guidelines on School Health Programme*. https://nhm.gov.in/New_Updates_2018/NHM_Components/RMNCHA/AH/guidelines/Operational_guidelines_on_School_Health_Programme_under_Ayushman_Bharat.pdf
8. Family Planning Division (2022). *India's Vision FP 2030*. Ministry of Health and Family Welfare. https://fp2030.org/sites/default/files/India_FP2030_Vision_Document.pdf
9. India budget. (2023). *Economic Survey 2022-23*. Ministry of Finance Department of Economic Affairs. <https://www.indiabudget.gov.in/economicsurvey/doc/echapter.pdf>
10. Indian Council of Medical Research, Public Health Foundation of India, and Institute for Health Metrics and Evaluation. (2017). *India: Health of the Nation's States - The India State-level Disease Burden*. [India_Health_of_the_Nation's_States_Report_2017.pdf \(healthdata.org\)](https://www.healthdata.org/india-health-of-the-nation-states-report-2017)
11. Sarwal, R., Prasad, U., Madangopal, K., Kalal, S., Kaur, D., Kumar, A., Regy, P., & Sharma, J. (2021). Investment Opportunities in India's Healthcare Sector. In *NITI Aayog*. https://www.niti.gov.in/sites/default/files/2021-03/InvestmentOpportunities_HealthcareSector_0.pdf

